

1. Administrative & Study Identification

**Full Study Title:** Phase 1/2 Open-label Study of BMS-986466 in Combination With Adagrasib With or Without Cetuximab in Participants With KRAS G12C-mutant Advanced Solid Tumors **Short Title/Acronym:** BMS-986466 + Adagrasib +/- Cetuximab in KRAS G12C Solid Tumors **Protocol Number:** CA126-0015 **NCT Number:** NCT06024174 **Trial Status:** TERMINATED (Study was terminated early) **Sponsor Name:** Bristol-Myers Squibb **Investigational Product:** BMS-986466, Adagrasib, and Cetuximab (Trade Name: Erbitux; ATC Code: L01FE01; EMA URL: [https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information\\_en.pdf](https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf)) **Phase of Development:** PHASE1, PHASE2 **Medical Monitor:** Bristol-Myers Squibb Clinical Operations Lead

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2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To find a safe, tolerable, and efficacious dose of BMS-986466 when given orally, in combination with adagrasib with or without cetuximab in participants with advanced KRAS G12C-mutant non-small cell lung cancer (NSCLC), pancreatic duct adenocarcinoma (PDAC), biliary tract cancer (BTC), or colorectal cancer (CRC). **Secondary Objectives:** To evaluate the Objective Response Rate (ORR), Duration of Response (DoR), Disease Control Rate (DCR), and pharmacokinetic profiles of the combination therapies.

**2.2 Background & Rationale** To provide a combination treatment option and improve outcomes in patients with advanced solid tumors harboring the KRAS G12C mutation. KRAS G12C mutations have historically been difficult to treat, and combination therapies are needed to overcome resistance pathways frequently observed with single-agent KRAS G12C inhibitors.

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3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Control Type:** Single-arm / Sequential Assignment (Dose Escalation and Expansion) **Blinding:** Open-label **Randomization:** N/A

**3.2 Planned Enrollment & Duration Target \$N\$:** 5 (Actual enrollment due to the study being terminated early) **Number of Sites:** Global, multi-center **Estimated Study Start:** 09-Nov-2023 (Actual) **Estimated Primary Completion:** 13-May-2024 (Actual)

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## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Confirmed diagnosis of advanced KRAS G12C-mutant NSCLC, CRC, PDAC, or BTC that has spread to other parts of the body and cannot be removed surgically. 2. For NSCLC and CRC: Documented KRAS G12C mutation status from an FDA/NYS approved or CE marked test, or positive status confirmed by a central laboratory. For PDAC and BTC: Documented KRAS G12C mutation from an approved test (blood samples for retrospective testing). 3. Part 1: Relapsed or refractory to available standard of care treatments. Part 2: Have not received previous treatment with KRAS inhibitors (NSCLC/CRC) and have failed, had disease recurrence, or are unable to tolerate after at least 1 previous line of therapy.

**4.2 Exclusion Criteria** 1. Tumors with known BRAF V600X, PTPN11, or KRAS Q61X mutations. 2. Any significant heart disease or condition. 3. Receiving concurrent medications that are strong CYP3A4 substrates, inducers, or inhibitors.

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## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** Specified dose on specified days for BMS-986466 combined with Adagrasib, with or without Cetuximab. **Route of Administration:** Oral (BMS-986466 and Adagrasib) and Intravenous Infusion (Cetuximab). **Duration of Treatment:** Until disease progression, unacceptable toxicity, or study termination.

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care medications for adverse events. **Prohibited:** Strong CYP3A4 substrates, inducers, and inhibitors.

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## 6. Study Timeline & Visit Schedule

| Visit ID     | Window (Days)                                      | Key Activities/Procedures                                              |
|--------------|----------------------------------------------------|------------------------------------------------------------------------|
| Screening    | Day -28 to 0                                       | Informed Consent, Medical History, KRAS G12C Biomarker Testing         |
| Baseline     | Day 0                                              | Enrollment, First Dose, Baseline Tumor Assessment                      |
| Interim      | Cycle 1 Day 15, 18, and Day 1 of subsequent cycles | Safety Labs, PK/PD sampling, DLT monitoring, Tumor/Efficacy Assessment |
| End of Study | Upon progression/toxicity                          | Final Vital Signs, Exit Interview, IP Accountability                   |

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## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoints** \* **Number of Participants With Dose Limiting Toxicity (DLTs):** Incidence of DLTs (including Grade 3/4 neurotoxicity, CRS, AST/ALT elevations, etc.) to determine the Maximum Tolerated Dose (MTD). \* **Number of Participants With**

**Adverse Events (AEs):** Any new untoward medical occurrence or worsening of a preexisting condition. \* **Number of Participants With Serious Adverse Events (SAEs):** Any untoward occurrence resulting in death, life-threatening conditions, permanent disability, or inpatient hospitalization.

**7.2 Secondary & Exploratory Endpoints \* Pharmacokinetics (PK):** Maximum Observed Plasma Concentration (C<sub>max</sub>), Time to Maximum Concentration (T<sub>max</sub>), and Area Under the Serum Concentration-time Curve From Time Zero to the Time of the Last Quantifiable Concentration (AUC[0-T]). \* Objective Response Rate (ORR) per RECIST 1.1. \* Duration of Response (DoR) and Disease Control Rate (DCR) per RECIST 1.1.

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## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Continuous monitoring of untoward medical occurrences including neurotoxicity, seizures, and Cytokine Release Syndrome (CRS). **Serious Adverse Events (SAEs):** Reported within 24 hours; includes any event resulting in death, life-threatening conditions, permanent disability, or inpatient hospitalization. **Data Monitoring Committee (DMC):** Internal Safety Review Committee assessing DLTs during the dose-escalation phase.

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## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Safety Set (all participants receiving \$\\ge 1\$ dose); Efficacy Set (evaluable participants with target lesions). **Sample Size Justification:** Adaptive dose-escalation design (similar to modified toxicity probability interval/TITE-BOIN design) to determine MTD. **Handling of Missing Data:** Actual times used for pharmacokinetic analyses; non-evaluable participants replaced as needed for DLT assessment periods.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote source data verification by the sponsor or designated CRO. **Audit Trail:** Validated electronic data capture (EDC) system with audit trails and data clarification forms.

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## 11. Study Identifiers & Governance

**Primary ID:** CA126-0015 **Registry Number:** NCT06024174 **Posting ID:** 417653501687391126 **Sponsor/Company:** Bristol Myers Squibb **Study Title:** A Study of BMS-986466 With Adagrasib With or Without

Cetuximab in Participants With Kirsten Rat Sarcoma Virus Glycine 12 to Cysteine (KRAS G12C)-Mutant Solid Tumors **Official Regulatory Title:** Phase 1/2 Open-label Study of BMS-986466 in Combination With Adagrasib With or Without Cetuximab in Participants With KRAS G12C-mutant Advanced Solid Tumors

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12. Clinical Framework (Study Specific)

**Disease Areas:** Advanced Solid Tumors **Preferred UMLS Name:** KRAS G12C-Mutated Solid Tumors **Disease Name:** Advanced Solid Malignancies **Preferred Name:** Advanced Malignant Solid Neoplasm **Semantic Type:** Neoplastic Process **Definition:** A malignant solid neoplasm that has spread extensively to other anatomic sites or is no longer responding to treatment. **SNOMED ID:** 39958004 **SNOMED Hierarchy:** 785317006|Advanced solid malignant neoplasm|Advanced solid malignant neoplasm (disorder) **Medical Methodology:** Combination Targeted Therapy **Optimization Target:** Maximum Tolerated Dose (MTD) & Anti-Tumor Efficacy

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13. Study Procedures & Methodology

**Management Pathway:** Standard oncology pathway for advanced solid tumors and dose-escalation **Education Protocol (HCP):** Investigator training on protocol, DLT assessments, and dose-modifications **Education Protocol (Patient):** Informed consent and oral medication dosing adherence **Quality Audit Mechanism:** Site monitoring and central laboratory verification for mutations

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14. Observation & Follow-Up Schedule

**Total Duration:** Until disease progression, toxicity, or study termination **Onsite Visit Cadence:** Cycle 1 Days 1, 15, 18; Day 1 of subsequent 28-day cycles **Remote Monitoring Frequency:** As specified per protocol and AE tracking **Checklist Review Interval:** Tumor assessments continuously scheduled per RECIST 1.1 guidelines

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15. Sign-off & Approval

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|--------------------------|--------|-----------|-------------|-----|------|-----|-----|
| Role                     | Name   | Signature | Date        | :-- | :--  | :-- | :-- |
| Principal Investigator   | [Name] | _____     | 03-Apr-2026 |     |      |     |     |
| Clinical Operations Lead | [Name] | _____     | 03-Apr-2026 |     | Lead |     |     |
| Statistician             | [Name] | _____     | 03-Apr-2026 |     |      |     |     |